

APO-CITALOPRAM TABLETS 20 MG

Product Description

White, oval shaped, biconvex, film-coated tablets, scored and engraved “2” bisect “0” on one side, “APO” on the other side.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Biochemical and behavioural studies have shown that citalopram is a potent inhibitor of the serotonin (5-HT)-uptake. Tolerance to the inhibition of 5-HT uptake is not induced by long-term treatment with citalopram.

Citalopram is the most Selective Serotonin Reuptake Inhibitor (SSRI) yet described, with no, or minimal, effect on noradrenaline (NA), dopamine (DA) and gamma aminobutyric acid (GABA) uptake.

The main metabolites of citalopram are all SSRIs although their potency and selectivity ratios are lower than those of citalopram. However, the selectivity ratios of the metabolites are higher than those of many of the newer SSRIs. The metabolites do not contribute to the overall antidepressant effect. In humans citalopram does not impair cognitive (intellectual function) and psychomotor performance and has no or minimal sedative properties, either alone or in combination with alcohol.

Pharmacokinetic properties

Absorption:

Absorption is almost complete and independent of food intake (T_{max} average/mean 3.8 hours). Oral bioavailability is about 80%.

Distribution:

The apparent volume of distribution (V_d) β is about 12.3 L/kg. The plasma protein binding is below 80% for citalopram and its main metabolites.

Biotransformation

Citalopram is metabolized to the active demethylcitalopram, didemethylcitalopram, citalopram-N-oxide and an inactive deaminated propionic acid derivative. All the active metabolites are also SSRIs, although weaker than the parent compound. Unchanged citalopram is the predominant compound in plasma.

Elimination

The elimination half-life ($T_{1/2\beta}$) is about 1.5 days and the systemic citalopram plasma clearance (Cl_s) is about 0.33 L/min, and oral plasma clearance (Cl_{oral}) is about 0.41 L/min.

Citalopram is excreted mainly via the liver (85%) and the remainder (15%) via the kidneys. About 12% of the daily dose is excreted in urine as unchanged citalopram. Hepatic (residual) clearance is about 0.35 L/min and renal clearance about 0.068 L/min.

Indications

Treatment of depression and prevention of relapse/recurrence.

Panic disorders with or without agoraphobia.

Obsessive-compulsive disorder (OCD).

Recommended Dosage

Posology

Treating Depression

Citalopram should be administered as a single oral dose of 20 mg daily. Depending on individual patient response this may be increased to a maximum of 60 mg daily. The dose may be taken in the morning or evening without regard for food.

A treatment period of at least 6 months is usually necessary to provide adequate maintenance against the potential for relapse.

Treating Panic Disorder

In common with other pharmacotherapy used in this patient group, a low starting dose is advised to reduce the likelihood of a paradoxical initial anxiogenic effect. A single oral dose of 10 mg daily is recommended for the first week before increasing the dose to 20 mg daily. The dose may be further increased, up to a maximum of 60 mg daily dependent on individual patient response.

Maximum effectiveness of citalopram in treating panic disorder is reached after about 3 months and the response is maintained during continued treatment. Dependent on individual patient response it may be necessary to continue treatment for several months.

Obsessive-Compulsive Disorder: Adults: Initially 20mg, may be increased to a maximum of 60mg.

Elderly patients

The recommended daily dose is 20 mg. Dependent on individual patient response this may be increased to a maximum of 40 mg daily.

Children

Not recommended, as safety and efficacy have not been established in this population.

Mode of Administration

Oral route

Contraindications

Hypersensitivity to citalopram.

Monoamine Oxidase Inhibitors: Cases of serious and sometimes fatal reactions have been reported in patients receiving an SSRI in combination with monoamine oxidase inhibitor (MAOI), including the selective MAOI selegiline and the reversible MAOI

(RIMA), moclobemide and in patients who have recently discontinued an SSRI and have been started on a MAOI.

Some cases presented with features resembling serotonin syndrome. Symptoms of a drug interaction with a MAOI include: hyperthermia, rigidity, myoclonus, autonomic instability with possible rapid fluctuations of vital signs, mental status changes that include confusion, irritability and extreme agitation progressing to delirium and coma. Citalopram should not be used in combination with a MAOI. Citalopram may be started 14 days after discontinuing treatment with an irreversible MAOI and at least one day after discontinuing treatment with the reversible MAOI (RIMA), moclobemide. At least 7 days should elapse after discontinuing citalopram treatment before starting a MAOI or RIMA.

Warnings

Suicidality in Children and Adolescents

- Antidepressant increase the risk of suicidal thinking and behavior (suicidality) in children and adolescents with major depressive disorder (MDD) and other psychiatric disorder.
- Anyone considering the use of antidepressant in a child or adolescent for any clinical use must be balance the risk of increased suicidality with the clinical need
- Patients who are started on therapy should be observed closely for clinical worsening, suicidality or unusual changes in behavior
- Families and caregivers should be advised to closely observe the patient and to communicate with the prescriber
- This product is not indicated for paediatric use.

Diabetes – In patients with diabetes, treatment with an SSRI may alter glycaemic control, possibly due to improvement of depressive symptoms. Insulin and/or oral hypoglycaemic dosage may need to be adjusted.

Seizures – Seizures are a potential risk with antidepressant drugs. The drug should be discontinued in any patient who develops seizures. Citalopram should be avoided in patients with unstable epilepsy and patients with controlled epilepsy should be carefully monitored. Citalopram should be discontinued if there is an increase in seizure frequency.

ECT – There is little clinical experience of concurrent administration of citalopram and ECT, therefore caution is advisable.

Mania – Citalopram should be used with caution in patients with a history of mania/hypomania.

Citalopram should be discontinued in any patient entering a manic phase.

Suicide – As improvement may not occur during the first few weeks or more of treatment, patients should be closely monitored during this period. The possibility of a suicide attempt is inherent in

depression and may persist until significant therapeutic effect is achieved and it is general clinical experience with all antidepressant therapies that the risk of suicide may increase in the early stages of recovery.

Haemorrhage – There have been reports of cutaneous bleeding abnormalities such as ecchymoses and purpura, as well as haemorrhagic manifestations e.g. gastrointestinal haemorrhage with SSRIs. The risk of gastrointestinal haemorrhage may be increased in elderly people during treatment with SSRIs. Caution is advised in patients taking SSRIs, particularly in concomitant use with drugs known to affect platelet function (e.g. atypical antipsychotics and phenothiazines, most tricyclic antidepressants, aspirin and nonsteroidal anti-inflammatory drugs (NSAIDs)) as well as in patients with a history of bleeding disorders.

Experience with citalopram has not revealed any clinically relevant interactions with neuroleptics. However, as with other SSRIs, the possibility of a pharmacodynamic interaction cannot be excluded. Consideration should be given to factors which may affect the disposition of a minor metabolite of citalopram (didemethylcitalopram) since increased levels of this metabolite could theoretically prolong the QTc interval in susceptible individuals. As with most antidepressants, citalopram should be discontinued if the patient enters a manic phase. There is little clinical experience of concurrent use of citalopram and ECT.

Some patients with panic disorder experience an initial anxiogenic effect when starting pharmacotherapy. A low starting dose reduces the likelihood of this effect.

Sexual dysfunction

- Selective serotonin reuptake inhibitors (SSRIs)/serotonin norepinephrine reuptake inhibitors (SNRIs) may cause symptoms of sexual dysfunction. There have been reports of long-lasting sexual dysfunction where the symptoms have continued despite discontinuation of SSRIs/SNRIs.

Interaction with other medicinal products

Simultaneous administration of citalopram and MAOIs may cause serotonin syndrome.

A pharmacokinetic interaction study of citalopram and metoprolol has shown a 2-fold increase in metoprolol concentrations. There was no significant increase in the effect of metoprolol on blood pressure and heart rate in healthy volunteers by adding citalopram.

Cimetidine cause a moderate increase in the average steady-state level of citalopram. Caution should be exercised at the upper end of the dose range of citalopram when it is used concomitantly with high doses of cimetidine.

There is no interaction with lithium and alcohol, and no clinically important interactions with levomepromazine and imipramine.

Pregnancy and lactation

Use of citalopram in pregnant women is limited. Reproduction toxicity studies have not given evidence of an increased incidence of fetal damage or other deleterious effects on the reproductive process.

Lactation – Citalopram is known to be excreted in breast milk. It is estimated that the sucking infant will receive about 5% of the weight related maternal daily dose (mg/kg). No or only minor events have been observed in the infants. However, the existing information is sufficient for assessment of the risk to the child. Caution is recommended.

Observational data indicate an increased risk (less than 2-fold) of postpartum haemorrhage following SSRI/SNRI exposure within the month prior to birth.

Undesirable effects

Adverse effects observed with citalopram are in general mild and transient. They are most prominent during the first one or two weeks of treatment and usually attenuate as the depressive state improves.

The most commonly observed adverse events associated with the use of citalopram and not seen at an equal incidence among placebo-treated patients were: nausea, somnolence, dry mouth, increased sweating and tremor. The incidence of each in excess over placebo is low (<10%).

In exceptional cases, seizures have occurred. Citalopram may cause a small reduction in heart rate which normally is without clinical importance.

However, in patients with preexisting low heart rate this may lead to bradycardia.

Overdose

Symptoms

Somnolence, coma, stiffened expression, episode of grand mal convulsion, sinus tachycardia, occasional nodal rhythm, sweating, nausea, vomiting, cyanosis, hyperventilation.

No case was fatal. The clinical picture was inconsistent.

Treatment

There is no specific antidote. Treatment is symptomatic and supportive. Gastric lavage should be carried out as soon as possible after oral ingestion. Medical surveillance is advisable.

Storage Conditions

Store below 30°C.

Availability of Dosage Forms

Citalopram 20 mg Tablets are packaged in a PVC/PVdC blister pack size of 30's.

Manufacturer

Apotex Inc-Etobicoke Site
50 Steinway Boulevard
Etobicoke, Ontario
Canada M9W 6Y3.

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Apotex Inc.

150 Signet Drive, Weston
Ontario, Canada M9L 1T9

Distributor : Pharmaforte (M) Sdn Bhd